



P46 · 40 PAGES, PRINTABLE + FILLABLE

Eldercare, the First 90 Days

A forty-page planner for the family member who has just had a parent or partner's situation change. The stroke. The fall. The dementia diagnosis. The hospital discharge to "now what." Three months of structure for the medical team, the legal paperwork, the where-they-will-live conversation, the siblings, and the caregiver's own collapse. With pages that name what most planners skirt: the grief, the resentment, the burnout.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

A note before you start

Something has changed.

Your mom had a stroke. Your dad was diagnosed with dementia. Your partner fell. The hospital discharged them with a list of follow-up appointments and a quiet "we'll see." Whichever way it landed in your house, three months from now, the new normal will have been built.

This planner is for those three months. The first month is mostly shock and logistics. The second month is the long stretch where the appointment schedule becomes routine and you start to feel the weight. The third month is when the family's pattern stabilizes, for better or for worse.

You are not the patient. You are the person who has been recruited, sometimes without warning, into being the primary witness of someone else's diminishment. That is a real job. It is mostly unpaid. It is rarely acknowledged.

This planner is the structure. It does not tell you which decisions to make. It is the place to write them down, and the questions to ask of the team who is helping you make them.

With care, Maya

This is a planning and tracking tool, not medical or legal advice. The clinical decisions in your loved one's care belong to them, their medical team, and you and your family acting on their behalf. Financial and legal decisions belong to you, your family, and the financial planner or attorney working with you. The planner does not name medications. It does not recommend a specific living situation. It is the surface where the conversation gets recorded.

How to use this planner

- Print on plain paper. Bring the medical-team page to the first appointment so the names are recorded.
- Some pages are for you alone. Some are meant to be filled in with a sibling or partner. The page labels say which.
- The crisis-line page is at the back. Bookmark it now. Caregivers are at elevated risk of burnout, depression, and worse. The line is there for them.
- The “what they want to remember” page is optional. It is for legacy work, if you and your loved one have the bandwidth and want to.

The first call list

Day one. Whatever happened, the calls you need to make.

THE PATIENT

Name: _____
Date of birth: _____
Their address (current): _____
Where they are right now: _____
(hospital, rehab, home, etc.)

THE MEDICAL TEAM (fill in as you go)

Primary care provider: _____
Number: _____
The specialist coordinating this: _____
Number: _____
Hospital: _____
Hospital social worker: _____
(sometimes the most important person)
Discharge planner: _____

THE LEGAL PEOPLE (fill in as you find them)

Their attorney (if any): _____
Their financial planner (if any): _____
Their tax accountant: _____

THE FAMILY (the call list)

Sibling 1: _____
Sibling 2: _____
Sibling 3: _____
Their spouse / partner: _____
Their best friend: _____
Their pastor / clergy: _____

WHO IS THE PRIMARY POINT OF CONTACT FROM THE FAMILY

This will get asked a lot. Pick a person. Often the geographically closest, or the one with the most flexibility.

Primary contact name: _____
Backup contact name: _____

Day 1

DATE: _____

WHAT HAPPENED, IN MY OWN WORDS

WHO I HAVE TOLD SO FAR

WHO I STILL NEED TO TELL

WHAT THE MEDICAL TEAM SAID TODAY

WHAT THE NEXT 48 HOURS LOOK LIKE

THE THING I AM CARRYING TONIGHT THAT I HAVE NOT TOLD ANYONE

Day 2 through 7, the first week

DAY 2 DATE: _____

Where they are: _____

The call I made today that mattered:

What the team told me:

How I am:

DAY 3 DATE: _____

The call:

The update:

How I am:

DAY 4 DATE: _____

The call:

The update:

How I am:

DAY 5 DATE: _____

The call:

The update:

How I am:

DAY 6 DATE: _____

The call:

The update:

How I am:

DAY 7 DATE: _____

ONE-WEEK NOTE TO MYSELF:

What is real now that was not real seven days ago:

What is most urgent for week 2:

Week 2

DATES: _____ to _____

WHERE THE PATIENT IS NOW

- ☐ Still in the hospital
- ☐ In rehab / skilled nursing facility
- ☐ At home, alone
- ☐ At home, with help
- ☐ At my house
- ☐ Other: _____

THE TEAM CONVERSATIONS THIS WEEK

Provider 1: _____
What they said: _____

Provider 2: _____
What they said: _____

Discharge / social worker: _____
What they said: _____

THE QUESTIONS I STILL HAVE

1. _____
2. _____
3. _____
4. _____

THE FAMILY THIS WEEK

Who has called or visited:

Who I am angry at:

(You are allowed to be angry. Write it.)

Who has surprised me by showing up:

MY OWN BANDWIDTH

Hours of sleep, average: _____

Meals eaten properly: _____

Days I cried: _____

The hardest moment this week:

Week 3

DATES: _____ to _____

THE BIG QUESTIONS THIS WEEK

Are we trending toward a permanent change in their living situation?

- ☐ Yes, clearly
- ☐ Yes, but I cannot say it out loud yet
- ☐ Maybe
- ☐ No, this is temporary

What is the team saying about the prognosis?

What am I starting to believe is true, that I did not believe at the start of this?

PAPERWORK STATUS

- ☐ Power of Attorney (POA): in place / being drafted / not yet
- ☐ Advance directive / living will: in place / being drafted / not yet
- ☐ Healthcare proxy: in place / not yet
- ☐ HIPAA release for me to receive info: in place / not yet
- ☐ Will / estate plan: in place / being updated / unclear
- ☐ Bank accounts: I have visibility / I do not
- ☐ Insurance: I have the policy info / I do not

Week 4, the first month closes

DATES: _____ to _____

WHAT IS DIFFERENT NOW THAN AT WEEK 1

WHAT IS THE SAME

THE PATIENT'S CURRENT FUNCTIONAL STATUS (in plain words, not medical)

What they can do without help:

What they need help with:

What they used to do that they no longer can:

MY CURRENT ROLE IN THEIR CARE

- ☐ Primary caregiver (it is mostly me)
- ☐ Coordinating but not hands-on
- ☐ Shared with siblings
- ☐ Backed by paid help
- ☐ A mix
- ☐ Something else: _____

THE THING I AM PROUDEST OF, FROM THIS MONTH

THE THING I REGRET MOST OR DID NOT DO WELL

(You are doing this in real time, with no training. Be kind.)

Week 5

DATES: _____ to _____

THE RHYTHM IS STARTING TO SETTLE

The weekly appointment schedule:

Home help (if any): _____

When they come: _____

What they do: _____

What I am still doing myself that someone else could do:

THE GUILT CHECK

What I am feeling guilty about, that I should not be:

What I should perhaps be feeling guilty about, but am not:

Week 6

DATES: _____ to _____

THIS WEEK'S MEDICAL UPDATES

THIS WEEK'S NON-MEDICAL CHALLENGES

WHAT I AM ASKING FOR HELP WITH THIS WEEK (specifically)

Week 7

DATES: _____ to _____

A MID-PERIOD HONESTY CHECK

Am I still functioning? ☐ Yes ☐ Barely ☐ No

Am I sleeping? ☐ Yes ☐ Some ☐ No

Am I eating? ☐ Yes ☐ Some ☐ No

Am I in physical pain? ☐ No ☐ Yes (where: _____)

Am I crying daily? ☐ No ☐ Yes

Am I drinking more than usual? ☐ No ☐ Yes

Have I seen my own doctor recently? ☐ Yes ☐ No, I should

The hardest part right now:

Week 8

DATES: _____ to _____

TWO MONTHS IN. HOW THE PATIENT IS, IN PLAIN WORDS

HOW I AM, IN PLAIN WORDS

WHAT I WANT THE NEXT MONTH TO LOOK LIKE

Weeks 9, 10, 11, 12

WEEK 9 DATES: _____ to _____

This week's update:

WEEK 10 DATES: _____ to _____

This week's update:

WEEK 11 DATES: _____ to _____

This week's update:

WEEK 12 DATES: _____ to _____

This week's update:

Week 13, the 90-day check-in

The closing week of this planner. By now, you have a pattern. Maybe a good one. Maybe a hard one.

DATES: _____ to _____

THE 90-DAY ASSESSMENT

WHERE THE PATIENT IS

Living situation: _____

Functional status: _____

Medical situation: _____

Mental status: _____

WHERE I AM

How am I doing, on a 1-10 scale: ____

How is my marriage / partnership (if applicable): ____

How is my own work / job: ____

How is my own health: ____

How is my relationship with my siblings (if applicable): ____

WHAT IS WORKING

WHAT IS NOT

WHAT NEEDS TO CHANGE IN THE NEXT 90 DAYS

- ☐ The care arrangement (where they live, who is helping)
- ☐ The family workload split
- ☐ My own work situation
- ☐ My own mental health (therapist, group, time off)
- ☐ The medical team (different specialist, second opinion)
- ☐ The financial / legal paperwork
- ☐ Something else: _____

THE LETTER TO MYSELF, NINETY DAYS FROM TODAY

Dear future me, in ninety days,

What I hope is true:

What I am willing to fight for:

What I am willing to let go of:

The medical team page

A reference page. Fill in as you meet each person. Bring this page to every appointment.

PRIMARY CARE

Name: _____
Phone: _____
Office address: _____
The role: _____

SPECIALIST 1 (the one coordinating)

Name: _____
Phone: _____
Office address: _____
The role (cardiology / neuro / oncology / etc.):

SPECIALIST 2

Name: _____
Phone: _____
The role: _____

SPECIALIST 3

Name: _____
Phone: _____
The role: _____

HOSPITAL TEAM (if currently inpatient)

Hospitalist: _____
Charge nurse: _____
Social worker: _____
(this is the person who often makes
the discharge plan)
Case manager: _____

HOME HEALTH / OUTPATIENT

Visiting nurse: _____
Home health aide: _____
Physical therapist: _____
Occupational therapist: _____
Speech therapist: _____

PHARMACY

Pharmacist who knows the case: _____
Pharmacy phone: _____

HOSPICE (if relevant)

Hospice team contact: _____

Hospice nurse: _____

The medication log (by purpose only)

The planner does not name medications. The medical team names them. The pharmacist confirms them. The log records what they prescribed and what it is for, in plain words.

PURPOSE (what is the medication for)	ROUTE	TIMES PER DAY
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____

ANY DIETARY OR INTERACTION NOTES THE PHARMACIST GAVE ME

WHEN TO CALL THE TEAM (their specific instructions, written down)

THE PRESCRIBER FOR EACH MEDICATION

(Fill in next to each entry above. If the answer is "I do not know," that is a question for the next appointment.)

Financial + legal paperwork

This page is the checklist. Each item routes to a professional. You are gathering, not drafting.

DOCUMENTS TO CONFIRM OR PUT IN PLACE

☐ DURABLE POWER OF ATTORNEY (POA)

Who is the POA: _____

Date executed: _____

Where the original is kept: _____

Attorney who drafted it: _____

☐ HEALTHCARE PROXY / MEDICAL POWER OF ATTORNEY

Who is named: _____

Date executed: _____

Where the original is kept: _____

☐ ADVANCE DIRECTIVE / LIVING WILL

In place: ☐ yes ☐ no

Where the original is kept: _____

☐ HIPAA RELEASE (so I can speak with their medical team)

In place: ☐ yes ☐ no

Where the original is kept: _____

☐ WILL OR TRUST

In place: ☐ yes ☐ no

Updated recently: ☐ yes ☐ no ☐ unclear

Attorney who drafted it: _____

FINANCIAL VISIBILITY (do I have access to or knowledge of)

- ☐ Their primary bank accounts
- ☐ Their retirement / 401k / pension
- ☐ Their Social Security
- ☐ Their Medicare / Medicaid status
- ☐ Their long-term-care insurance (if any)
- ☐ Their home insurance / mortgage status
- ☐ Their tax filings (last year's return)
- ☐ Their bills + recurring payments
- ☐ Their safety deposit box (location + key)
- ☐ Their digital accounts (email, social, online banking) - logins
- ☐ Their vehicle title

THE ATTORNEY I AM WORKING WITH

Name: _____
Phone: _____
Specialty (elder law preferred): _____

THE FINANCIAL PLANNER I AM WORKING WITH (if any)

Name: _____
Phone: _____

A NOTE: ELDER-LAW ATTORNEYS SPECIFICALLY HANDLE THIS TYPE OF SITUATION AND ARE OFTEN WORTH THE INVESTMENT EVEN IF YOUR LOVED ONE HAS A GENERAL ATTORNEY. ASK FOR REFERRALS FROM THE SOCIAL WORKER, FROM OTHER FAMILIES IN SIMILAR SITUATIONS, OR FROM YOUR LOCAL BAR ASSOCIATION'S REFERRAL SERVICE.

The “where they will live” decision page

This is one of the heaviest decisions in the 90 days. The planner does not tell you what to choose.

THE OPTIONS YOU MAY BE WEIGHING

STAY IN THEIR OWN HOME, WITH HELP

Pros: Familiar. Their stuff. Their neighborhood.

Cons: Logistically intensive. May require home modifications.

May not be safe long-term. Costly home-health.

MOVE IN WITH ME (OR ANOTHER FAMILY MEMBER)

Pros: Closer eyes on them. Saves on facility costs.

Cons: Caregiver burnout. Marriage strain. Loss of their autonomy. Loss of your home being yours.

ASSISTED LIVING

Pros: Independence for them. Social environment.

Some support without medical level.

Cons: Cost. Adjustment period. Not appropriate for high-medical-needs.

SKILLED NURSING FACILITY (long-term care)

Pros: Medical staff on site. Appropriate for high needs.

Cons: Cost. Loss of autonomy. Quality varies widely.

MEMORY CARE (for dementia)

Pros: Specialized for memory disorders.

Cons: Cost. Often locked unit. Adjustment.

HOSPICE (when appropriate)

Pros: Comfort-focused. Medicare/insurance covered.

Care can happen at home or in facility.

Cons: Requires accepting that prognosis is limited.

WHAT WE ARE WEIGHING

The patient's preference (in their words, if they can tell you):

The medical team's recommendation:

Our financial reality:

What the siblings think (each, separately):

What feels right to me, separately from all of the above:

DECISION (when made):

Option chosen:

Date decided:

Move date (if applicable):

Cost estimate (monthly):

A NOTE: THIS DECISION MAY BE REVISITED. THE FIRST CHOICE IS NOT ALWAYS THE FINAL CHOICE. PEOPLE'S NEEDS CHANGE. THE FACILITIES PEOPLE GO TO FROM HOME ARE OFTEN A STEPPING STONE, NOT A FINAL PLACE. GIVE YOURSELF PERMISSION TO RE-EVALUATE IN A FEW MONTHS.

The siblings + family page

Sometimes the family closes ranks. Sometimes the family splinters. Often it does both, on the same week.

THE FAMILY WORKLOAD SPLIT (in plain words, no diplomacy)

What I am doing:

What sibling 1 is doing:

What sibling 2 is doing:

What sibling 3 is doing:

What no one is doing, that should be:

THE HARD CONVERSATION I HAVE NOT HAD YET, AND WITH WHOM

THINGS I AM ANGRY ABOUT, RELATED TO MY FAMILY

THINGS A SIBLING HAS DONE THAT I AM GRATEFUL FOR (even if I am also angry, both are allowed)

OLD WOUNDS THAT ARE RESURFACING

Family dynamics from when you were younger often come back during eldercare. The favored child. The black sheep. The one who left. The one who stayed. None of those identities get to drive the

current decisions, but they all show up in the room.

What old patterns I am noticing:

What I want to NOT do, this time around:

A FAMILY THERAPIST OR ELDER-FAMILY MEDIATOR may be worth the investment if the conversation cannot move without one. They exist. The social worker can refer you.

The “I am not okay either” page

Caregiver collapse is well-documented. Caregivers of aging parents have higher rates of depression, anxiety, sleep disorders, marital strain, and their own health issues. The planner names this honestly.

THE HONEST CHECK

How many hours of sleep, average, this past week: _____
How many real meals (not just coffee + snacks): _____
How many days I cried this week: _____
How many days I drank more than usual: _____
How many days I yelled at someone I love: _____
How many appointments of my own I have missed: _____
How many friends I have stopped texting: _____

How much of my own time is currently mine, in hours per week:
____ hours

Is that enough? ☐ Yes ☐ No

WHAT I WOULD TELL A FRIEND IN MY POSITION TO DO

WHY I AM NOT DOING THAT MYSELF

THE PROVIDER I AM RESERVING THE NUMBER FOR

My own primary care: _____
A therapist (specialty: family / caregiving / grief): _____
A caregiver support group: _____

CAREGIVER CRISIS / SUPPORT LINES

US, 988 Suicide and Crisis Lifeline

US, Eldercare Locator: 1-800-677-1116
(Connects to local Area Agency on Aging, they have caregiver support, respite, and more)

US, Family Caregiver Alliance: 1-800-445-8106

US, Alzheimer's Association 24/7 Helpline: 1-800-272-3900
(For dementia-related caregiver support specifically. they
also help with non-Alzheimer's dementias)

UK, Carers UK: 0808 808 7777

Canada, Carers Canada: contact via website

A reminder: caregiver burnout is a leading cause of nursing-home
placement. The data is clear. Taking care of yourself is not
selfish. It is what keeps you in this for the long haul.

The “what they want to remember” page

Optional. For the legacy work, if you and your loved one have the bandwidth and want to.

If your loved one can still talk with you about their life, this page is the place to capture some of it. Not in a "we are documenting them because they are dying" way. In a "I would like to remember this when you are gone" way.

QUESTIONS TO ASK (one per visit, when the moment is right)

1. What is the earliest memory you have?

2. Who was the most important person in your life when you were young?

3. What is something you are proud of that I might not know about?

4. What is a regret you would tell a younger version of yourself about?

5. What did you love about your mother / father?

6. What did you find hard about them?

7. What is one piece of advice you want me to remember after?

8. Is there anyone you want to make peace with before you can't?

9. What did you wish you had told me, that you never have?

10. What do you want at your service, if and when?

A note: you do not have to do this all in one visit. Or any of it. Some people want to do this work. Some do not. Take your cue from your loved one. If they want to talk, take notes. If they do not, do not force it. The legacy work is theirs to consent to.

A final letter

You made it through ninety days.

You met the team. You wrangled the paperwork. You sat in waiting rooms. You answered the same questions from siblings five times. You had at least one cry in a parking lot.

The next ninety days will be different. The shock has settled. The patterns are set. The decisions you made may need to be revisited. Some will be wrong. You will make different ones.

What this planner cannot do is tell you that everything will be okay. Sometimes it is. Sometimes the loved one stabilizes and a new long chapter begins. Sometimes things continue to decline. Sometimes the loss is closer than the planner can address.

What the planner can do is hold the structure. The medical team, the paperwork, the conversations, the decisions. So that on whichever day in whichever year you look back, you have a record.

You are doing the work that most of us will do for someone, eventually. There is dignity in it. There is also exhaustion in it. Both are allowed to be true.

If the next chapter is ongoing caregiving, P50 Family Caregiver Workbook is the next planner. If the chapter is moving toward end-of-life, the local hospice team has the most specific support and is usually covered by insurance.

You are not alone in this, even when it feels like you are.

With care, Maya



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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